

العمر(تاريخ الميلاد) : 27-09-1982 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقر بما فيه التوقيع: (-----)

خطه العلاج : أبحاث أورام رنين علي الفقرات العنقيه والصدرية بالصبغه [11112]

التعليق على التشخيص : -----

الاسم: د/محمد كيلاي
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: هند سيد عبدالعزيز محمد National ID: 28209270102087	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي MRI cervical and thoracic spine with contrast
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: أورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Left breast malignant looking mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- Ki67: Positive in 17% of tumor cells.
- SISH: Amplified.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr.Amal Hareedy.

Dr.Rana Fathy.

Verified by :

Dr.Toka Medhat



Name: <u>هند سيد عبدالعزيز محمد</u>		
MR. : <u>1945</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has metastatic breast cancer with bone involvement and is receiving HERCEPTIN IV and ZOMETA in addition to FEMARA; an echocardiogram performed on 3/2 showed an ejection fraction of 64%, and a new CT scan of the chest, abdomen, and pelvis was pending.		
The patient continued on FEMARA alone on the day of assessment, with HERCEPTIN IV and a repeat echocardiogram scheduled; a new CT scan of the chest, abdomen, and pelvis was still pending.		
The patient received HERCEPTIN IV, FEMARA, and ZOLADEX today, with the next ZOLADEX dose planned for 16-08; an echocardiogram will be performed at the next visit.		
Clinical pharmacy intervention documented that the patient had received HERCEPTIN and FEMARA the previous week and received ZOMETA today.		
A new echocardiogram was requested, and a bone scan was ordered; the patient continues on HERCEPTIN IV and FEMARA.		
The patient with metastatic breast cancer to bone is on paracetamol 1000 mg every 6 hours, and tramadol 50 mg every 12 hours was added for pain control.		
The patient remains on FEMARA and HERCEPTIN for metastatic breast cancer to bone; the most recent bone scan compared with April 2026 shows stable disease, and the CT CAP was free of progression, allowing continuation of the current treatment regimen.		
تحريراً في ,		
التاريخ :	18-09-2026	الوقت :
الطبيب المعالج :	هنا اسامه عبد الرحيم عبد البر	
يعتمد ,		



Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast malignant looking mass.

Nature of Specimen: Tru-cut biopsy, collected at 29/12/2024.

Gross Examination:

Four grayish white tissue cores up to 1 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, SISH, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Undetected.
- * Perineural invasion: Undetected.
- * Angioinvasion: Undetected.
- * Microcalcifications: Undetected.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 40% of tumor cells (score 7/8).
 - Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
 - Ki67: 17% nuclear reaction of tumor cells.
 - SISH: Amplified.

(Average Her2 copy No. (6.2), Her2/CEP17 ratio: (3.3))

Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.

Diagnosis:

Verified by :

Dr.Toka Medhat

Printing Date : 28/01/2025 12:58:38

Page 1 of 3

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذة / هند سيد عبد العزيز محمد	Request No.	: 13009837
Patient No.	: 28209270102087	Request Date	: 30/12/2024
Path No.	: 5251/24	Verification Date	: 28/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 42 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Dr.Samia Ali.

Verified by :
Dr.Toka Medhat

Printing Date : 28/01/2025 12:58:38

Page 3 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Mrs. HEND SAED ABDELAZIZ
Date: 10-12-2025

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast cancer, underwent NACT , follow ups study.

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry suspicious microcalcifications.
- UOQ focal asymmetry , likely at site of previous surgical drainage
- Benign calcifications
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- UIQ ill-defined spiculated mass lesion.
- Evidence of benign macrocalcification.
- **Ultrasound** demonstrated scattered simple cysts.
- 11-12 o'clock, zone b, 9 mm from skin, ill- defined hypoechoic spiculated mass lesion, measures 15 x 10 mm, with peripheral color uptake on doppler study.
- 12 o'clock, zone a small hypoechoic circumscribed nodule, measures 6 x 3 mm, with no color uptake on doppler study.
- A small 11 o'clock, zone a, rounded lesion measures 3 x 2 mm, with slight echogenic contents, likely complicated cyst
- Normal skin/nipple/areola complex.
-No pathological axillary, infra or supra clavicular lymph nodes.

Conclusion: -

Right breast (BIRADS 2): Bening macrocalcifications, post-surgical drainage sequelae

Left breast (BIRADS 6): 11-12 o'clock suspicious mass lesion, with 12 o'clock indeterminate nodule and 11 o'clock? complicated cyst.

Best Regards,
DR. RANIA AHMED, MD

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

PATIENT NAME: HEND SAYED ABDEL AZIZ MOHAMMED

GENDER: Female

DATE : 15-Apr-2026 11:49 AM

PATIENT ID: 10125259772

AGE: 43 y

ORDER: 3829337

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report: Compared to previous bone scan dated 2/3/2025, the current study revealed :-

More sclerosis of the still seen suspicious DV10 lesion.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

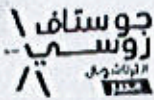
More or less stationary course of the DV10 lesion.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

15-Apr-2026 12:31 PM





Patient's Name:

HIND SAYED ABDEL AZIZ

Date:

2026-04-04

Referred by:

Prof.Dr. CAP+C

Hospital:

Goustave Roussey International Egypt

Opinion:

❖ D10 vertebral body sclerotic area and left iliac bone tiny lytic focus are seen for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Patient's Name: HIND SAYED ABDEL AZIZ
Date: 2026-04-04
Referred by: Prof.Dr. CAP+C
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA with no available previous films for comparison.

I- CHEST:

- Left basal atelectatic band.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Gall bladder shows a 21 mm stone.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D10 vertebral body predominantly sclerotic area is noted.
- Tiny left iliac bone lytic focus is seen.



Name: Hend Sayed Abd El-Aziz.

Date: 15.6.2025 .

Referred by Prof. DR. Mohammed Hefny.

MRN : 30192747.

F18-FDG PET/CT STUDY

CLINICAL HISTORY:

- * 42 y-old female patient of breast cancer.

REASON FOR PET/CT STUDY:

- * Assessment of response to therapy.

TECHNIQUE:

- Radiopharmaceutical: 18F FDG.
- Time from 18F-FDG injection to scan: 60 min
- PET/CT images were acquired from the skull base through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and sagittal planes.
- Additional notes: SUV max normalized to body weight was used.

COMPARISON: No previous PET CT studies.



57357

مؤسسة مستشفى سرطان
الأطفال - مصر
Children's Cancer Hospital
Foundation - Egypt

Nuclear Medicine Dept.
PET/CT Unit

Musculoskeletal:

- * D10 shows mixed lytic/sclerotic osseous lesion of SUVmax~3.5.

IMPRESSION:

No definite metabolically active FDG avid gross breast lesions.
FDG avid D10 bone lesion.

Thanks for referral;
Nuclear Medicine Unit;

Dr. Mary Rabea , MD
Radiology consultant

Dr. Maha Mehesen, MD.
Nuclear medicine consultant.

1 Fostat street, El Sayda Zainab, Cairo
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فاكس: (202) 25 35 1745



هند سيد عبدالعزيز محمد

DOB: 1962-09-27

Male

MRN: 289/2025

T FORM

24/12/2025 G

Name	هند عبدالعزيز محمد	MRN	289/2025
Age	42	ID	CNC-9C-V-01-CAN
Menopausal status	Pre	Phone	011 41411 8701
NRS SCORE		Marital status	married
BMI	>30	Family history	
CO morbidity	no	REASON OF MDT	Surgery-

Clinical Data

BX:				R		L	
ER: 818	PR: 718	HER 2: +2 amplified	Ki67: 17%				
Sonomammography: by SJH BSM → 1+ breast masses VOC							
Mets Workup: + axillary L-N. CT CAP → HfLs → Triphasic → benign Dio lesion → RIF Started Taxol / Herceptin 20mca - 34 weeks				TNM 3 1 1 (D10)			
Recommendations: PET/CT 6RS → almost CR last 12/11/2025 PET/CT 25/11/2025 → CR Continued Taxol / Herceptin BSM → Requested For MDT regarding Surgical Option if possible and Holding AT / Zoladex							
MDT OUTCOME & PLAN							
Continue Herceptin 1-2 + Zoladex 3/12/2025. as pt not fit for stat surgery. No SBT plan cancelling Pt about to start surgery							

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
H. Kachy	O. M. 24/12/2025	N. 24/12/2025			